Fax cover sheet

Mesa Verde Medical Center (fictional)

Case Management Department

FAX TRANSMITTAL

To: Illumination Health + Home - Recuperative Care Intake
Fax: 555-0199
From: Ruben Castellanos, MSW
Phone: 555-0160
Date: 07/17/2026 08:30
Pages (including cover): 12
Re: Recuperative care referral
Urgent [X] For Review [X] Please Reply [X]

Message

63F CAP + COPD, improving, completing PO abx.
Target d/c 07/17 PM once home O2 is set up. H&P, progress notes, RT and PT notes attached.

CONFIDENTIALITY NOTICE

This facsimile contains confidential health information. If received in error, notify sender and destroy.

-- FICTIONAL DEMO RECORD - not a real patient --

IHH Recuperative Care Referral Form

ILLUMINATION HEALTH + HOME

Recuperative Care Referral Form (RC0725-SH)

Referring Party

Hospital: Mesa Verde Medical Center (fictional)
Department/Floor: 4 North Medical
Social Worker/Case Manager/Nurse: Ruben Castellanos, MSW
Phone: 555-0160
Email: rcastellanos@mesaverde.example
Nursing Station: 555-0161
Authorized By: Dr. J. Whitcombe

Patient Information

Name: Linda Doe
MRN: MV-1186430
Birth Date: 09/27/1962
Gender: F
Ethnicity: Asian
SSN (last 4): 0000
Insurance: Orange Coast Health Plan (fictional) Medi-Cal
Preferred Language: English
Housing Situation:
Currently Unhoused [X]
4+ episodes of homelessness in the past 3 years [ ]

Hospital Admission Details

Chief Complaint/Admitting Diagnosis: Community-acquired pneumonia, COPD
Admit Date: 07/11/2026
Any Wounds: Yes [ ] No [X]
Number/Location/Size/Stage:
Independent with Wound Care: Yes [ ] No [ ]
Home Health Ordered: Yes [ ] No [X]
PPD/TB Test or Chest X-Ray: Date: 07/11/2026 Result: CXR RLL infiltrate, improving

History of Health & Living Situation

O2: Yes [ ] No [X]
Self Administer All Meds: Yes [X] No [ ]
Continent of Bowel & Bladder: Yes [X] No [ ]
Colostomy/Ileostomy: Yes [ ] No [X]
Foley Catheter: Yes [ ] No [X]
Diabetes: Yes [ ] No [X]
Insulin: Yes [ ] No [X]
Diabetes Oral Meds: Yes [ ] No [X]
Active Infection: Yes [ ] No [X]
Ambulatory: Yes [X] No [ ]
Assistive Device: Yes [ ] No [X]
Hospice: Yes [ ] No [X]
Car: Yes [ ] No [X]
Service Animal: Yes [ ] No [X]

Behavioral and Mental Health Status

Any limitations or challenges: No [X] Yes [ ]

Invented for the Claude Impact Lab demo. Names, facilities and numbers are fictional.

CalAIM Community Supports authorization request

CalAIM COMMUNITY SUPPORTS - AUTHORIZATION REQUEST

Service: Recuperative Care (Medical Respite)
Health Plan: Orange Coast Health Plan (fictional) - Medi-Cal

Member

Member Name: Linda Doe
DOB: 09/27/1962
Member ID: OCH-40927315

Referring Provider

Facility: Mesa Verde Medical Center (fictional)

Clinical

Primary Diagnosis: Pneumonia, unspecified organism
ICD-10: J18.9
Member is experiencing homelessness [X]
Member at risk of hospitalization/re-hospitalization [X]
Requested Start Date: 07/17/2026
Requested Days: 14
Secondary Dx: COPD with acute lower respiratory infection (J44.0)

Plan Use Only

Status: Pending review

-- FICTIONAL DEMO RECORD - not a real patient --

Patient face sheet

Mesa Verde Medical Center (fictional) - PATIENT FACE SHEET

Patient: Linda Doe
MRN: MV-1186430
Account #: 99186430
DOB: 09/27/1962
Sex: Female
Preferred Language: English
Ethnicity: Not Hispanic or Latino
Marital Status: Single
Address: NO FIXED ADDRESS (homeless)
Mailing: 1234 Example Ave, Santa Ana, CA 92700 (shelter mail drop)
Phone: 555-0100 (message)

Encounter

Admit Date/Time: 07/11/2026 22:10
Admit Type: Inpatient
Unit/Bed: 4N-409
Attending: Julian Whitcombe, MD

Insurance

Primary: Orange Coast Health Plan (fictional) - Medi-Cal
Subscriber: Self

Emergency Contact

None listed

-- FICTIONAL DEMO RECORD - not a real patient --

History and Physical - page 1

HISTORY AND PHYSICAL

Date: 07/11/2026

CC

Fever, productive cough, worse SOB x 4 days

HPI

63 y/o F with COPD (GOLD 2 per 2024 PFTs), current smoker 1/2 ppd, homeless sleeping in her tent near Example Park x 2 years, presents with 4 days of fever, rusty sputum and dyspnea. No home O2 at baseline. EMS SpO2 85% RA. Uses albuterol inhaler PRN, ran out 1 month ago.

PMH

- COPD
- GERD

Meds PTA

- Albuterol HFA PRN (out x 1 month)
- Omeprazole 20 mg daily

Allergies

NKDA

Social

Homeless, tent. Smokes 1/2 ppd. EtOH denies. Drugs denies.

-- FICTIONAL DEMO RECORD - not a real patient --

History and Physical - page 2

Exam

T 101.6F HR 114 BP 118/70 RR 26 SpO2 86% RA -> 93% on 3L NC
Gen: ill-appearing, speaking full sentences
Lungs: RLL crackles and bronchial breath sounds, scattered expiratory wheeze
CV: tachy, regular
Neuro: AOx4

Data

WBC 17.9, lactate 1.6, procalcitonin 1.8
CXR: RLL consolidation, no effusion, no pneumothorax
COVID/flu/RSV PCR negative

Assessment/Plan

63F with CAP RLL and COPD with acute exacerbation.
1. CAP - ceftriaxone 1 g IV daily + azithromycin 500 mg IV daily; blood cx x2, sputum cx
2. AECOPD - duonebs q4h, prednisone 40 mg PO daily x 5 days. Titrate O2 to SpO2 88-92%. No BiPAP needed.
3. Tobacco - nicotine patch 14 mg, counseling
4. Dispo - RT, PT; CM for recuperative care
Code: Full. DVT ppx: heparin 5000 units SC q8h
Signed: Julian Whitcombe, MD 07/11/2026 23:40

-- FICTIONAL DEMO RECORD - not a real patient --

Hospitalist progress note

HOSPITALIST PROGRESS NOTE - HD#6

Date: 07/16/2026 07:50
S: Cough improving, sputum now clear. No fevers x 72h. Walking in hall with RT.
O: T 98.5F HR 88 BP 122/74 RR 18 SpO2 91% RA at rest; 86% RA with ambulation -> 92% on 2L NC
Lungs: faint RLL crackles, minimal wheeze
Blood cx 07/11 x2: no growth, final. Sputum cx: normal flora.
A/P:
- CAP RLL improving. Transitioned 07/14 to cefpodoxime 200 mg PO BID to complete 7 days total (last dose 07/17 PM). Azithromycin course complete 07/15.
- COPD - prednisone course complete 07/15. Tiotropium + albuterol MDI.
- New exertional and nocturnal hypoxemia: qualifies for home O2 (see RT). Order home oxygen 2 L/min NC with exertion and sleep.
- Medically stable for d/c 07/17 once home O2 arranged
Signed: Julian Whitcombe, MD

-- FICTIONAL DEMO RECORD - not a real patient --

Respiratory therapy note

RESPIRATORY THERAPY NOTE

Date: 07/16/2026 09:30
Breath sounds: diminished RLL, scattered end-exp wheeze, improved from admission

Home O2 Qualification Walk

SpO2 RA at rest: 91%
SpO2 RA with ambulation 200 ft: nadir 86%
SpO2 on 2L NC with ambulation: 92-93%
Overnight oximetry 07/15: SpO2 <88% for 41 min on RA
Meets criteria for home O2 with exertion and sleep. Not required at rest.
Treatments: duonebs stopped 07/15. Tiotropium 18 mcg daily + albuterol MDI PRN with spacer, technique correct on teach-back.
Pt educated on O2 safety, no smoking near oxygen; verbalized understanding.
Signed: A. Bernal, RRT

-- FICTIONAL DEMO RECORD - not a real patient --

Invented for the Claude Impact Lab demo. Names, facilities and numbers are fictional.

Physical therapy evaluation

PHYSICAL THERAPY EVALUATION

Date: 07/15/2026 11:00
Dx: CAP, COPD, deconditioning

Prior Level of Function

Independent ambulation without device.

Assessment

Bed mobility: Independent
Sit<>stand: Independent
Transfers: Independent
Gait: 200 ft without device, modified independent, 1 standing rest break, on 2L NC portable O2
Stairs: not tested
Balance: static good, dynamic good

Plan / Recommendation

Safe for d/c to recuperative care. Will need portable O2 for walking. No DME needs other than O2.
Signed: K. Ostrowski, PT

-- FICTIONAL DEMO RECORD - not a real patient --

Case management note

CASE MANAGEMENT NOTE

Date: 07/16/2026 10:40
Pt AOx4, agreeable to recuperative care. Independent with ADLs per nursing.
Home O2 order (2 L/min NC with exertion and sleep, concentrator + portable tanks) faxed to Pacific Breeze Medical Supply (fictional), 555-0167, on 07/16/2026 10:15.
Awaiting vendor confirmation of delivery date and delivery address.
PCP follow-up: not yet scheduled; pt to establish with clinic after d/c.
Signed: Ruben Castellanos, MSW

-- FICTIONAL DEMO RECORD - not a real patient --

Vitals and labs

VITAL SIGNS - LAST 5 SETS

Date/Time | Temp (F) | HR | BP | RR | SpO2 | O2
07/15 08:00 | 98.6 | 92 | 124/76 | 18 | 91% | RA
07/15 16:00 | 98.4 | 90 | 120/72 | 18 | 92% | RA
07/16 00:00 | 98.1 | 84 | 118/70 | 16 | 93% | 2L NC (sleep)
07/16 07:50 | 98.5 | 88 | 122/74 | 18 | 91% | RA
07/16 12:00 | 98.3 | 86 | 120/74 | 16 | 92% | RA

LABS

07/11 | WBC 17.9 | Hgb 13.2 | Plt 342 | Cr 0.8 | Procalcitonin 1.8
07/15 | WBC 10.6 | Hgb 12.7 | Na 138 | K 4.2 | Cr 0.7 | Glu 104

MICROBIOLOGY

07/11 Blood cx x2: No growth, final
07/12 Sputum cx: Normal respiratory flora
07/12 MRSA nares PCR: Negative
07/17 05:30 SARS-CoV-2 rapid antigen: Negative (added 07/17 AM before d/c)

IMAGING

07/11 CXR: RLL consolidation
07/15 CXR: improving RLL opacity, no effusion or pneumothorax

-- FICTIONAL DEMO RECORD - not a real patient --

Discharge medication list

DISCHARGE MEDICATION LIST

Patient: Linda Doe MRN: MV-1186430

- Cefpodoxime 200 mg PO BID - last dose 07/17 PM
- Tiotropium 18 mcg inhaled daily (NEW)
- Albuterol HFA 90 mcg 2 puffs q4-6h PRN SOB, with spacer
- Omeprazole 20 mg PO daily
- Nicotine patch 14 mg/24h daily

DME / Respiratory

- Home oxygen 2 L/min via nasal cannula with exertion and during sleep
- Spacer

Prednisone and azithromycin courses complete.

Patient able to self-administer all medications: Yes

30-day supply dispensed from outpatient pharmacy: Yes

-- FICTIONAL DEMO RECORD - not a real patient --

Invented for the Claude Impact Lab demo. Names, facilities and numbers are fictional.